[redacted]

[redacted] Master [redacted]

[redacted]

2024062570844 SHEET

Patient's Name ..... PIMS No .....

Date & Time	
15/3/26 7:00am	C/S/B – [redacted]
	Δ = ENTERIC FEVER
DO Antibiotics ⇓ 5.	C/o fever spike yesterday @ 10:45pm – 102.5°F
	C/o Cough (+)
	No C/o Irritability
	No C/o Lethargy
	No C/o Altered Sensorium
⇒ Oral intake improved.	No C/o Abdominal pain
	No C/o Vomiting
	No C/o Headache
⇒ No further loose stools	O/E: Alert, Active, Afebrile
	PR: 108/min
⇒ No features of decreased urine output	RR: 22/min
	SpO2: 98% @ Room Air
	S/E:
No c/o headache	P/A: Soft, non tender, Liver is palpable 4 ↓ RCM
No c/o vomiting	Liver Span: 10.5cm
	CNS: No focal neurological deficit
No c/o bone pain	CVS: S1S2 (+)
No c/o burning Micturition	RS: B/L AE (+)
No features of hepatomegaly.	Plan: – [redacted]
No features [redacted]	1) Continue Ceftriaxone
	2) To Send CBC with PS, CRP in Next Prick.
	3) W/F Complications

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
15/03/06 5.00 pm	S/B [redacted]
	A = ENTERIC FEVER
	child reviewed,
	C/o - fever spike @ 12:30pm today - 102.2°F
	No new Complaints
	O/E: Child is Alert, Active, Afebrile
	PR: 100/min
	RR: 22/min
	SpO2 - 98% @ Room Air
	S/E...
	P/A: Soft, non tender, Liver palpable 4cm ↓ RCM
	Liver Span: 10.5cm
	RS: B/L AE (+)
	CNS - No focal neurological deficit
	CVS - S1S2 (+)
	Plan:-
	1) Continue Ceftriaxone
	2) To Send CBC with PS, CRP in Next Prick.
	3) To Send H1N1 tommorow
	[redacted]

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
16/3/26 1:00pm	CLSIB – [redacted]

Δ = ENTERIC FEVER

C/o Fever Spike @10pm Yesterday – 100.8°F
No C/o Cough
No C/o Nasal discharge
No C/o Altered Sensorium
No C/o Abdominal pain
No C/o Vomiting. No C/o Headache.

O/E:- Alert/Active,
PR : 108/min
RR : 24/min
SpO2 : 98% @ Room Air

S/E:-
P/A :- Soft, non tender, Liver palpable 4cm ↓ RCM
Liver Span : 10.5cm
CNS: No focal neurological deficit
CVS: S1S2 (+)
RS: B/L AE (+)

Plan:-
1) Continue Ceftriaxone
2) To Send CBC with PS & CRP + Widal Pack
3) To Collect H1N1 Swab Reports

[redacted]

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
16/8/24 9:30am	CCS/B - [redacted]
	Child Reviewed,
	C/o fever spike at 10pm yesterday - 100.8°F
	No new Complaints
	O/E: Child is Alert, Active, Afebrile
	PR - 100/min
	RR - 22/min
	SpO2 - 98% @ Room Air
	S/E:-
	P/A: soft, non tender, Liver is palpable 1cm ↓RCM
	Liver Span - 10.5cm
	RS: B/L AE (+)
	CNS - NFND
	CVS: S1S2 (+)
	Plan:-
	1) Continue Ceftriaxone
	2) To Send CBC with PS, CRP in next prick
	3) W/F Complications

[redacted]